

RIVERGATE REGIONAL TRAUMA CENTER

Level I Trauma Center
2100 Medical Center Drive, Rivergate, CA 91766

Reyes, Yolanda

MRN: RRTC-0074318 DOB: 06/22/1991 (34 yrs) Sex: F
Adm: 10/18/2025 Compiled: 11/17/2025

OPERATIVE REPORT COMPILATION

Patient: Reyes, Yolanda
MRN: RRTC-0074318
DOB: 06/22/1991 (34 yrs)
Sex: F
Admitted: 10/18/2025
Attending of Record: Priya Ramaswamy, MD · Trauma Surgery
Compilation printed: 11/17/2025

Contents: three operative reports encompassing the admission of 10/18/2025 through 11/03/2025.

1. 10/18/2025 · Emergent Right Frontotemporoparietal Decompressive Hemicraniectomy, Evacuation of Acute Subdural Hematoma, and Intracranial Pressure Monitor Placement (Neurosurgery)
2. 10/21/2025 · Open Reduction and Internal Fixation, Pelvic Ring (Orthopedic Trauma Surgery)
3. 11/03/2025 · Open Reduction and Internal Fixation, Bilateral Tibia/Fibula (Orthopedic Trauma Surgery)

OPERATIVE REPORT · 1 of 3

Date of Procedure: 10/18/2025
Time in / out of room: 05:40 / 08:55
Location: Trauma OR 2

PREOPERATIVE DIAGNOSIS

1. Severe closed traumatic brain injury, Glasgow Coma Scale 6 on arrival, mechanism blunt head trauma (pedestrian struck by transit bus).
2. Right frontotemporoparietal acute subdural hematoma with 9 mm midline shift, CT-documented.
3. Elevated intracranial pressure with early uncal herniation signs (right pupil sluggish, 4 mm).

POSTOPERATIVE DIAGNOSIS

Same as above, status post decompression and hematoma evacuation.

PROCEDURES PERFORMED

1. Right frontotemporoparietal decompressive hemicraniectomy.
2. Evacuation of acute subdural hematoma.
3. Expansile duraplasty with pericranial graft.
4. Placement of right frontal intraparenchymal intracranial pressure monitor.

PERSONNEL

Surgeon: Aaron Feldstein, MD · Neurosurgery

First Assistant: Priya Ramaswamy, MD · Trauma Surgery

Anesthesia: General endotracheal anesthesia (patient intubated in the field, ETT already in place); balanced

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anesthesia with fentanyl, midazolam, and low-dose isoflurane titrated to intracranial pressure and hemodynamics.

Anesthesiologist: Diane Kowalski, MD

ANESTHESIA RECORD

General endotracheal anesthesia was induced without incident. Given the preoperative Glasgow Coma Scale of 6 and the extent of intracranial injury, the patient remained intubated and mechanically ventilated throughout the procedure and postoperatively. On emergence, the patient did not regain purposeful responsiveness; post-anesthesia and post-operative neurologic exam was notable for persistent depressed consciousness (GCS 7T, sedated/intubated), consistent with the severity of the underlying traumatic brain injury rather than any anesthetic complication.

FINDINGS AND TECHNIQUE

A large right frontotemporoparietal question-mark incision was made and a bone flap approximately 12 x 15 cm was removed. The underlying dura was tense and bulging. Durotomy was performed with evacuation of approximately 60 mL of acute subdural hematoma and underlying contused, non-viable brain tissue. Hemostasis was achieved with bipolar cautery and hemostatic agents. An expansile duraplasty was fashioned using a pericranial graft. The bone flap was banked in a subcutaneous pocket in the left lower abdominal wall for anticipated delayed cranioplasty. A right frontal bolt was placed and an intraparenchymal intracranial pressure monitor advanced, with an opening pressure of 28 mmHg. The scalp was closed in layers over a subgaleal drain.

INTRAOPERATIVE DATA

Estimated Blood Loss: 250 mL.

Complications: None intraoperatively.

Disposition: Transferred intubated and sedated to the Neuro-ICU; ventilator-dependent and unresponsive to voice at the time of this report.

OPERATIVE REPORT · 2 of 3

Date of Procedure: 10/21/2025
Time in / out of room: 09:10 / 12:35
Location: Trauma OR 1

PREOPERATIVE DIAGNOSIS

1. Unstable open-book pelvic ring disruption (AO/OTA 61-C, APC-II pattern), status post external fixation on 10/18/2025.

POSTOPERATIVE DIAGNOSIS

Same as above, status post definitive open reduction and internal fixation.

PROCEDURES PERFORMED

1. Removal of anterior external fixator.
2. Open reduction and internal fixation, symphysis pubis (anterior plating).
3. Percutaneous reduction and internal fixation, right sacroiliac joint (iliosacral screws).

PERSONNEL

Surgeon: Marcus Alvarez, MD · Orthopedic Trauma Surgery

First Assistant: Delia Prasad, DO · PGY-4 Orthopedic Surgery Resident

Anesthesia: General endotracheal anesthesia; balanced anesthesia with standard induction and maintenance agents titrated to hemodynamics.

Anesthesiologist: Diane Kowalski, MD

ANESTHESIA RECORD

The patient was already intubated and mechanically ventilated from her ongoing neurosurgical/ICU course (see 10/18/2025 operative report) and remained so throughout this procedure without a separate induction. She tolerated the case without hemodynamic instability. Postoperatively she was returned to the Neuro-ICU still intubated and ventilator-dependent; she remained unresponsive to voice on post-anesthesia exam, consistent with her ongoing depressed level of consciousness from the traumatic brain injury and not attributable to any new intraoperative event.

FINDINGS AND TECHNIQUE

The prior anterior external fixator was removed. The symphysis pubis diastasis was reduced under direct visualization through a Pfannenstiel approach and secured with a six-hole reconstruction plate. Attention was then turned to the right posterior ring; the sacroiliac joint disruption was reduced under fluoroscopic guidance and secured with two 7.3 mm iliosacral screws. Final fluoroscopic images confirmed anatomic reduction and stable hardware position.

INTRAOPERATIVE DATA

Estimated Blood Loss: 400 mL.

Complications: None intraoperatively.

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Disposition: Returned intubated to the Neuro-ICU in stable condition.

OPERATIVE REPORT · 3 of 3

Date of Procedure: 11/03/2025
Time in / out of room: 07:50 / 11:20
Location: Trauma OR 1

PREOPERATIVE DIAGNOSIS

1. Left tibia/fibula shaft fractures, status post initial splinting.
2. Right tibia/fibula shaft fractures, status post initial splinting.

POSTOPERATIVE DIAGNOSIS

Same as above, status post definitive open reduction and internal fixation, bilateral.

PROCEDURES PERFORMED

1. Open reduction and internal fixation, left tibia, with intramedullary nail.
2. Open reduction and internal fixation, right tibia, with intramedullary nail.
3. Open reduction and internal fixation, right fibula, with plate and screws.

PERSONNEL

Surgeon: Marcus Alvarez, MD · Orthopedic Trauma Surgery

First Assistant: Delia Prasad, DO · PGY-4 Orthopedic Surgery Resident

Anesthesia: General endotracheal anesthesia; balanced anesthesia with standard induction and maintenance agents titrated to hemodynamics.

Anesthesiologist: Rania El-Sayegh, MD

ANESTHESIA RECORD

The patient tolerated induction and the procedure without hemodynamic instability. She remained ventilator-dependent postoperatively, consistent with her ongoing intensive-care course for the traumatic brain injury described in the 10/18/2025 operative report. Postoperative neurologic exam continued to show depressed consciousness consistent with the ongoing, gradual recovery phase of that injury, without any new focal deficit attributable to this procedure.

FINDINGS AND TECHNIQUE

Left leg: a transverse mid-shaft tibial fracture was reduced under fluoroscopic guidance and stabilized with a reamed intramedullary nail; the associated non-displaced fibular fracture was managed nonoperatively and did not require separate fixation. Right leg: a comminuted mid-shaft tibial fracture was reduced and stabilized with a reamed intramedullary nail; a short oblique distal-shaft fibular fracture was reduced and secured with a plate and screws through a separate lateral incision. All wounds were irrigated and closed in layers.

INTRAOPERATIVE DATA

Estimated Blood Loss: 300 mL, combined.

Complications: None intraoperatively.

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Disposition: Returned to the intensive care unit in stable condition.